

Ethics and Communication

Question set

Graham Steell edition · September 2026

32 questions, one per page.

Item numbers are the original numbering, so gaps are expected.

Answers are in the companion document.

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☐  Mark Question☐ Suggested Highlights

A 31-year-old woman comes to the emergency department for evaluation of nausea, vomiting, and abdominal pain over the past 2 days. The triage nurse notes that the patient's legal name differs from what was written on the intake form. While reviewing the patient's electronic medical record, you see that the patient's sex is listed as male and that multiple pronouns have been used to describe the patient in prior notes. In addition to introducing yourself to the patient, which of the following is the most appropriate initial statement?

- ☐ A. "I ask all my patients about their gender identity; could you please tell me if you identify as a man or a woman?" 
- ☐ B. "I like to ask my patients about the name they use and their pronouns; how would you like to be addressed?" 
- ☐ C. "I'd like to make sure that I refer to you appropriately; would you prefer that I use male or female pronouns?" 
- ☐ D. "I've reviewed your chart and wanted to be sure it's accurate; how would you describe your gender identity?" 
- ☐ E. "To be sure I ask pertinent questions for your symptoms, could you please verify your sex assigned at birth?" 

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Item: 2 of 40
Question Id: 11531



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☐ Mark Question

☐ Suggested Highlights

A 19-year-old woman comes to the clinic due to nausea and abdominal pain. She struggles to describe the pain and frequently points to her lower abdomen. The patient shakes her head no when asked if she could be pregnant or is sexually active. When she is asked about her last menstrual period, she nods affirmatively and says "now." She is single and immigrated to the United States from a country in South America several months ago. Physical examination shows moderate abdominal tenderness with no rebound or guarding and vital signs are stable. The patient is discharged home with a diagnosis of food poisoning. Later that day, she is taken by ambulance to the emergency department after experiencing severe abdominal pain followed by loss of consciousness. A diagnosis of ruptured ectopic pregnancy is made. Which of the following interventions would most likely have prevented the delay in diagnosis?

- ☐ A. Asking the patient whether she feels comfortable discussing sexual issues
- ☐ B. Assessing the patient's cultural health beliefs
- ☐ C. Assessing the patient's current social situation
- ☐ D. Calling family members to obtain additional history
- ☐ E. Determining whether the patient needs language assistance

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Item: 3 of 40
Question Id: 21678



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☐ Mark Question

☐ Suggested Highlights

A 37-year-old woman, gravida 2 para 1, at 8 weeks gestation comes to the clinic to establish prenatal care. The patient has had mild nausea, but no vomiting or vaginal bleeding. She has no chronic medical conditions and has had no surgeries. Her only medication is a prenatal vitamin. The patient's prior pregnancy ended in a spontaneous vaginal delivery at 38 weeks gestation. BMI is 32 kg/m². Physical examination is unremarkable. Ultrasound confirms an 8-week intrauterine pregnancy with normal cardiac activity. Although the patient's last delivery was at a hospital, she is now considering a planned home birth. Which of the following is the most appropriate statement to this patient?

- ☐ A. "A home birth can be very risky, and I don't think we should consider this option."
- ☐ B. "A home birth is fine as long as you have a good support system during the delivery."
- ☐ C. "Are there any specific aspects of your last delivery that concern you about a hospital delivery?"
- ☐ D. "As long as your pregnancy remains uncomplicated, I think that a home birth is a good option for you."
- ☐ E. "I think this is something we can consider, but let's readdress this when you are closer to your due date."

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Question Id: 21742

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☐  Mark Question

☐ Suggested Highlights

A 2-month-old girl is brought to the office for a well-child visit. The patient was born at term with no complications. She is exclusively formula fed and gaining weight well. Examination is unremarkable today. The mother is informed that several vaccinations are due at this visit. She becomes upset and mentions that her older son, who did follow a standard vaccine schedule, had a "bad reaction" to his last set of shots and had a cough for weeks. She wants to defer immunizations for her daughter at this time. The mother says, "I know you won't agree with me, but I have been reading that these reactions are actually very harmful to the immune system." Which of the following is the most appropriate response?

- ☐ A. "I understand your concern, and I believe that ensuring your children's health and safety is our shared goal." 
- ☐ B. "I want to reassure you that immunizations are safe, and we should proceed with vaccination today." 
- ☐ C. "I would like to provide you with a list of resources on vaccine safety so you can make the best decision for your child." 
- ☐ D. "I'm sorry that this is a stressful topic. Let's talk about this again at the next visit." 
- ☐ E. "This must be hard. I know you are concerned about vaccine side effects, but the risk of vaccine-preventable disease is far greater." 

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Item: 5 of 40
Question Id: 21171



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☐  Mark Question

☐ Suggested Highlights

A 57-year-old woman with a history of type 2 diabetes mellitus and asthma comes to the office for a preventive visit during influenza season. The patient belongs to an ethnic minority group and is accompanied by her daughter. The patient's medical conditions are well controlled, and she currently has no symptoms. The provider explains that she is at high risk for complications from seasonal influenza and recommends that she get the seasonal influenza vaccine. The patient says, "I don't trust that vaccine. It's from the government, so I don't think it's safe for people like me." Which of the following actions by the physician is most appropriate?

- ☐ A. Ask the patient's daughter to encourage her to get the vaccine 
- ☐ B. Assess the patient for additional delusional thoughts 
- ☐ C. Initiate discussion about the vaccine, incorporating culturally inclusive materials 
- ☐ D. Inquire about the patient's understanding of federally funded health care 
- ☐ E. Respect the patient's wishes and do not administer the vaccine 

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Item: 6 of 40
Question Id: 21178



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☐  Mark Question

☐ Suggested Highlights

A 66-year-old woman comes to the clinic for follow-up of sarcoidosis. Over the past year, her lung function has worsened significantly, and her probability of survival at 12 months is below 50%. When the physician suggests they talk about advance care planning and appointment of a surrogate decision maker, the patient replies, "Why would I want to sign away my life on a piece of paper? The Lord will take me when the time is right." Which of the following is the most appropriate response?

- ☐ A. "Although it can be difficult, it's important that you plan for end-of-life care." 
- ☐ B. "I understand that you may be scared, but this process is designed to protect you." 
- ☐ C. "Participating in this process will make it easier for your family to be involved in your care." 
- ☐ D. "This will help share your preferences for care if you lose the ability to communicate." 
- ☐ E. "You have the right to decline. I'm available to talk if you change your mind." 

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Question Id: 1432



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☐  Mark Question

☐ Suggested Highlights

A 62-year-old man comes to the office with his wife for evaluation of episodic chest pain. The physician introduces herself to the patient and his wife and confirms that he is comfortable and not experiencing any current pain. The patient's wife begins the conversation by saying, "He brought a list of his medications for you to review, but I'm not sure he takes all of them regularly." After thanking them for bringing the list, which of the following is the most appropriate way for the physician to initiate the medical interview?

- ☐ A. "Have you had trouble remembering to take your medications?" 
- ☐ B. "How will your pain improve if you don't take your medications?" 
- ☐ C. "May I ask your wife to leave for this part of the evaluation?" 
- ☐ D. "What do your episodes of chest pain feel like?" 
- ☐ E. "Why do you think you have difficulties taking your medications regularly?" 

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Question Id: 19317



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☐ Mark Question☐ Suggested Highlights

A 49-year-old woman comes to the office due to worsening hot flashes and night sweats over the past year. The patient's menstrual periods have been irregular for the past 2 years, and her last menstrual cycle was a year ago. She has no chronic medical conditions and has had no surgeries. BMI is 23 kg/m². Vital signs are normal. The neck is supple and without thyromegaly. The remainder of the examination is normal. The patient says, "This is awful. I turn red and sweat in the middle of work meetings and then have to leave to cool down. I'm wondering if hormones could help." Which of the following is the most appropriate response to this patient?

- ☐ A. "Although menopause can be uncomfortable, it is a natural process; let's try making some simple lifestyle changes first."
- ☐ B. "Hot flashes and sweating are very normal in menopause; for most women, they subside on their own in 1-2 years without treatment."
- ☐ C. "I can see these symptoms have been affecting your day-to-day life; hormonal therapy can be an effective option, although there are some risks."
- ☐ D. "I don't recommend taking hormonal therapy because your symptoms are not that severe; the risks would likely outweigh the benefits."
- ☐ E. "Menopause can be very stressful; perhaps talking about how to cope with the emotional effects would be helpful."

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☐  Mark Question

☐ Suggested Highlights

A 54-year-old man comes to the office for an initial appointment to evaluate lower back pain. The patient's medical record shows that he has seen several physicians for back pain treatment. He has been prescribed multiple pain medications and physical rehabilitation services but often stopped prematurely or did not follow through with recommendations. At the beginning of the visit the patient says, "I'm tired of living with back pain, and I haven't been able to work in years. Nothing has helped me—at this point I might as well just give up and go on disability." Which of the following statements is the most appropriate initial response to this patient?

- ☐ A. "Although previous medications have not worked, consultation with a pain specialist may be helpful." 
- ☐ B. "I understand the difficulty of your situation, but applying for disability will not improve your back pain." 
- ☐ C. "I understand your frustration, but it is difficult to help when you think nothing will work." 
- ☐ D. "It must be frustrating to have chronic back pain and feel like there are no options to ease your suffering." 
- ☐ E. "Your chances of improving would be better if you followed recommendations and remained in treatment." 

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☐  Mark Question

☐ Suggested Highlights

A 4-year-old girl is sent to the emergency department from her day care center due to fever, right ear pain, and repeated vomiting. Examination shows a bulging, cloudy tympanic membrane associated with erythema, swelling, and tenderness over the right mastoid. CT scan reveals opacification of the mastoid air cells with destruction of bony septa. When the patient's parents arrive, they report that the patient has had recurrent episodes of otitis media. However, the parents decline antibiotic therapy, stating, "In our family, we believe that natural healing techniques are better than shooting up our children with chemicals." Which of the following is the most appropriate response to the parents' statement?

- ☐ A. "Can you please tell me what you currently understand about your child's infection?" 
- ☐ B. "Help me understand what natural treatments you are willing to allow." 
- ☐ C. "I understand your concerns about medication side effects, but antibiotic treatment is necessary." 
- ☐ D. "If we don't give your child antibiotics, she could get a brain abscess and die." 
- ☐ E. "We can try natural techniques briefly, but if she doesn't get better, we will have to use antibiotics." 

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☐  Mark Question

☐ Suggested Highlights

An 18-year-old man comes to the office for a follow-up visit. He was seen in the emergency department 3 months ago because of panic attacks related to cocaine use. The patient subsequently stopped using cocaine and has had no recurrent anxiety symptoms. He graduated from high school, started a new job, and has been considering applying to community college. At today's visit, the patient reports that he used cocaine at a friend's house this past weekend. He says, "I really messed up. I feel like a failure." The patient has no history of illness. He is in no physical discomfort. Which of the following is the most appropriate statement by the physician at this time?

- ☐ A. "Do you think using cocaine again could have a negative impact on your job and going to college?" 
- ☐ B. "It's hard to quit. Let's go over the health effects of cocaine to help motivate you to stay sober." 
- ☐ C. "Quitting is difficult, so don't be so hard on yourself. Let's move forward from here." 
- ☐ D. "Socializing with your friend probably contributed to your relapse. Let's look at strategies to deal with peer pressure." 
- ☐ E. "You've been making real progress in quitting. What can we learn from this temporary setback?" 

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☐  Mark Question

☐ Suggested Highlights

A 29-year-old woman comes to the emergency department due to abdominal pain and fever. This morning, while crossing a street, she was struck in the stomach by a motorcyclist. On arrival, the patient tells the triage nurse that she does not have medical insurance. She has no chronic medical conditions. Temperature is 38 C (100.4 F), blood pressure is 109/82 mm Hg, and pulse is 88/min. The abdomen is distended with bruising, rebound tenderness, and guarding. The patient is told she needs exploratory surgery to treat her condition. She refuses and states angrily, "I'm suffering and I'm not going to be a guinea pig for this system. If I had insurance, you'd give me something that would actually cure me." In addition to clarifying the details of the surgical procedure with the patient, which of the following is the most appropriate response by the physician?

- ☐ A. "My compensation is the same regardless of your insurance status; I'm simply recommending the best treatment for you." 
- ☐ B. "I'm sorry you are suffering, but you could experience serious consequences if you refuse surgery." 
- ☐ C. "The insurance system can seem unfair. Would you like to discuss options with a social worker?" 
- ☐ D. "I want you to feel better. What can I do to help assure you that I only have your wellbeing in mind?" 
- ☐ E. "You seem to be in pain. Would you like some pain medication before we talk more?" 

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☐  Mark Question

☐ Suggested Highlights

A 54-year-old woman comes to the office for a yearly preventive examination. She has no specific concerns. Blood pressure is 144/95 mm Hg and pulse is 86/min. BMI is 50 kg/m². Examination shows severe obesity but is otherwise unremarkable. Fasting glucose is 128 mg/dL. When the subject of her weight is mentioned, the patient says, "In my family, we just like to eat. My mother is heavy, my father is heavy, and my kids are heavy. There's no point in trying to lose weight now." Which of the following is the most appropriate response to this patient's statement?

- ☐ A. "Bariatric surgery might be a good option for you to consider." 
- ☐ B. "Even with your family history, simple changes in your diet can lower your blood pressure." 
- ☐ C. "Tell me how you think your weight might relate to your blood pressure and glucose." 
- ☐ D. "You can still eat whatever you want as long as you don't eat so much of it." 
- ☐ E. "You don't have to lose all the weight at once. A little at a time will eventually make a difference." 

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☐  Mark Question

☐ Suggested Highlights

A 48-year-old man comes to the office with his wife for follow-up of major depressive disorder. The patient has been taking sertraline for 2 years, and due to a recurrence of depression, he started bupropion and weekly cognitive behavioral therapy 3 weeks ago. Since then, his energy level has improved, and he is working on communicating more with his wife about his emotions. The patient has no suicidal ideation and is hopeful that he will feel better with continued treatment. A week later, his wife calls the office and states that her husband died by suicide a few days ago. She is tearful on the phone and says, "I thought that he was doing better. I don't understand how this happened." Which of the following is the most appropriate response?

- ☐ A. "He seemed hopeful about his treatment when I last saw him; do you know whether he was taking his medication as prescribed?" 
- ☐ B. "I can't imagine how difficult this must be for you; I'm here to answer questions you may have about your husband's illness and treatment." 
- ☐ C. "I'm so sorry to hear about this; he seemed to be improving, but there can be a risk of suicidal thoughts with medication changes." 
- ☐ D. "I'm very surprised by this too, given how well he was doing at his last appointment; can you tell me more about what happened?" 
- ☐ E. "This is definitely upsetting and shocking, and I did not see this coming. Did you notice anything unusual the week leading up to his suicide?" 

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Question Id: 21977



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☐ Mark Question

☐ Suggested Highlights

A 52-year-old man comes to the office for a routine follow-up appointment. The patient has a history of well-controlled hypertension and hyperlipidemia. He has a 35-pack-year smoking history and reports that he continues to smoke about 10-15 cigarettes daily. Family history is significant for lung cancer in his mother. His father died following a stroke at age 65. When asked about whether he has considered quitting, the patient replies that he is not ready to stop smoking. He says, "I know I probably should, but I'm just too stressed at work to give up my cigarettes right now. To be honest, I'm probably smoking closer to a pack a day." Which of the following is the most appropriate response to the patient?

- ☐ A. "I suggest that you consider how your health may improve if you were to stop smoking."
- ☐ B. "I understand that you're under a lot of stress, but you are capable of more than you think."
- ☐ C. "What do you worry will happen if you quit smoking? What about if you don't quit?"
- ☐ D. "What is your understanding of how smoking relates to lung cancer and stroke?"
- ☐ E. "With all the smoking cessation treatment options available, quitting is a lot more manageable."

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☐  Mark Question

☐ Suggested Highlights

A 33-year-old man comes to the office due to back pain. He has a 2-week history of moderate pain in the lumbar area that started after a long day of yard work. There is no associated fever, motor weakness, or urinary symptoms. The patient has no history of acute trauma and medical history is unremarkable. Examination shows normal lower extremity strength, sensation, and reflexes. Straight-leg raising test is negative. The patient says, "My brother had similar pain last year and his MRI scan showed a bulging disc. Can you order that test for me?" The clinician offers options for symptomatic relief but recommends against performing any imaging studies. Which of the following is the most likely effect of this recommendation?

- ☐ A. Increased diagnostic uncertainty 
- ☐ B. Increased fragmentation of care 
- ☐ C. Increased health care disparity 
- ☐ D. Increased malpractice risk 
- ☐ E. Reduced clinical standardization 
- ☐ F. Reduced overutilization 

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Question Id: 11518

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☐  Mark Question

☐ Suggested Highlights

A 21-year-old man comes to the office due to recurrent ear drainage. He is accompanied by his girlfriend. He speaks in English. On examination, the patient has purulent otorrhea, a tympanic membrane perforation, and a pearly white mass behind the tympanic membrane consistent with a large cholesteatoma. He is told that he will need an operation on his ear to remove the mass. After listening to an explanation of the procedure and why it should be done, the patient nods briefly but does not ask questions. He is also given information sheets with a written description of the procedure and instructions on how to schedule surgery that he immediately hands to his girlfriend, who begins to read them. Review of the medical records shows he has been to the emergency department several times in the last year with this same concern and has not had a flu shot in the past 5 years. Which of the following is the most appropriate course of action?

- ☐ A. Ask him to schedule a follow-up appointment and bring a family member to help him make health care decisions 
- ☐ B. Explain again slowly and simply what surgery entails and why it should be done 
- ☐ C. Gently ask the patient to read the information sheet aloud to determine his literacy 
- ☐ D. Use a visual aid to educate the patient about the procedure 
- ☐ E. Verbally review instructions on how to schedule an appointment for the procedure 

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Question Id: 18762

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☐  Mark Question

☐ Suggested Highlights

A 38-year-old woman is admitted to the hospital overnight due to cellulitis. The next morning, the hospitalist on service introduces herself to the patient and asks how she is feeling. The patient angrily says, "It's almost lunchtime, and I still haven't gotten my breakfast. How am I supposed to get better when you're starving me?" The patient has an order to receive a regular diet. Which of the following is the most appropriate response by the physician?

- ☐ A. "Hunger can make anyone upset, so I'll come back after you've eaten." 
- ☐ B. "I apologize for the delay, but I'm sure your food will be here shortly." 
- ☐ C. "I'm sorry for the delay, but I don't control when meals are served, so please let your nurse know." 
- ☐ D. "Since I am here, let me examine you first and follow up on your breakfast afterward." 
- ☐ E. "You must be very hungry, so let me find out what happened to your meal." 

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Item: 26 of 40
Question Id: 21717



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☐  Mark Question

☐ Suggested Highlights

A 63-year-old man with a 30-pack-year smoking history is admitted to the hospital for evaluation of hemoptysis. Imaging studies reveal a right lung mass with features suggesting possible malignancy. On day 2 of hospitalization, he undergoes biopsy of the mass. The biopsy confirms the diagnosis of non-small cell lung cancer. However, staging studies have not yet been completed. As the physician enters the patient's room after reviewing the biopsy results, the patient asks about the biopsy results and prognosis. Which of the following is the best way to approach this patient's question?

- ☐ A. Defer discussion of both biopsy results and prognosis and tell him you will discuss them once all studies are completed 
- ☐ B. Defer discussion of both biopsy results and prognosis until an oncologist evaluates the patient 
- ☐ C. Discuss the biopsy results along with an optimistic assessment of prognosis until an unfavorable prognosis is confirmed with staging studies 
- ☐ D. Discuss the biopsy results with the patient, but defer discussion of prognosis until staging results are available 

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☐ Mark Question

☐ Suggested Highlights

A 27-year-old man comes to the office for follow-up regarding a herniated disc. He first injured the disc 3 months ago while at work. Nonsurgical treatment options, including physical therapy and as-needed NSAIDs and tramadol, were recommended. The patient has completed physical therapy, and follow-up MRI indicates that the herniated disc has regressed. He reports a significant amount of ongoing pain and asks for a dose increase and refill of his pain medication. In the past month, the patient has been to the emergency department on 2 occasions to request pain medication due to running out before his next scheduled refill. Vital signs are within normal limits, and physical examination is unremarkable. Which of the following statements by the physician would be most appropriate at this time?

- ☐ A. "Given your recent refill requests and interest in increasing your dose, I am concerned that you may be dependent on pain medication."
- ☐ B. "It doesn't seem like medication is effective anymore; let's talk about alternate ways to treat your pain."
- ☐ C. "Sometimes patients take pain medications to help with other types of symptoms; how has your mood been recently?"
- ☐ D. "You're still experiencing a lot of pain, and it has not been easy managing it; let's discuss how you have been taking the medication."
- ☐ E. "Your pain symptoms do not seem to be improving with additional medication, so a dose increase is unlikely to help you."

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Question Id: 10488

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☐  Mark Question

☐ Suggested Highlights

A 65-year-old man is admitted overnight to the hospital with abdominal pain. It is determined that he has an uncomplicated small bowel obstruction that will require surgery in the morning. Temperature is 37 C (98.6 F), blood pressure is 130/82 mm Hg, and pulse is 90/min. The resident meets with the patient and his adult son, who has accompanied him to the hospital, to discuss surgical intervention. After introducing herself, the resident learns that the patient speaks primarily Vietnamese and has limited English proficiency, although his son speaks both Vietnamese and English. The son shares that he is a critical care nurse and offers to interpret the conversation for his father. Which of the following responses to the patient's son is the most appropriate?

- ☐ A. "I appreciate your offer to interpret; however, I need to get a trained medical interpreter before I start talking with your father."
- ☐ B. "I'll print out the Vietnamese version of the consent form. It would be helpful if you could interpret any questions your father may have after reading it."
- ☐ C. "Thank you. That would be very helpful, especially since you are already familiar with medical terminology."
- ☐ D. "That would make things much easier; could you start by asking your father what his understanding of the situation is?"
- ☐ E. "That's a kind offer, but I wouldn't be able to have a direct and objective discussion with your father if you interpret it."

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Question Id: 19253



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☐ Mark Question

☐ Suggested Highlights

A 49-year-old man comes to the office due to dysuria and hematuria. Vital signs are within normal limits and physical examination is unremarkable; urinalysis shows gross hematuria but is otherwise normal. The physician discusses potential causes and the need for a cystoscopy. The patient is told what to expect during the procedure and is given written information and a referral list with instructions on how to schedule an appointment with a urologist. He anxiously glances over the papers, saying he will make an appointment soon. Office staff had informed the physician that the patient was unable to follow the directions on the sign-in sheet and declined to fill out paperwork. Which of the following physician responses is the most appropriate at this time?

- ☐ A. "I'd like to ask you some questions about your background; could you tell me how far you got in school?"
- ☐ B. "I'm concerned about your ability to read and fill out forms; have you always had trouble with this?"
- ☐ C. "Sometimes patients have difficulty making appointments; could you please read the instructions out loud?"
- ☐ D. "The forms you get before an appointment help me do a thorough evaluation; could you fill them out next time?"
- ☐ E. "We've discussed a lot of information today; how confident are you with scheduling an appointment for yourself?"

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Question Id: 10975



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☐  Mark Question

☐ Suggested Highlights

A 35-year-old man comes to the office for a routine health maintenance evaluation. After separating from his wife, the patient recently moved to "make a fresh start" and start a new job. He has no chronic medical conditions and has had no surgeries. The patient is sexually active and has had 6 lifetime partners. When completing the rest of the sexual history, which of the following questions is most appropriate for the physician to ask?

- ☐ A. "What are the genders of your current and previous sexual partners?" 
- ☐ B. "Can you tell me about your previous romantic relationships?" 
- ☐ C. "Did you have other sexual partners while you were married?" 
- ☐ D. "Do you identify as heterosexual, homosexual, or bisexual?" 
- ☐ E. "Have you ever had sexual intercourse with a man?" 

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☐  Mark Question

☐ Suggested Highlights

A 29-year-old woman comes to the office for evaluation of vaginal discharge and pruritus for the past week. The discharge is thick and yellow with no associated odor. The patient is sexually active with multiple partners. When asked about the genders of her sexual partners, the patient says, "Why does it matter? I'm pretty sure this is a yeast infection, so I just need some medication." Which of the following is the most appropriate response at this time?

- ☐ A. "I can see you're uncomfortable talking about this. We can discuss it at a follow-up appointment when you're ready." 
- ☐ B. "I routinely ask my patients about sexual partners because it helps me fully understand what their health needs are." 
- ☐ C. "It may be difficult to offer the most appropriate screening and treatment unless you are open about your sexual partners." 
- ☐ D. "This can be an uncomfortable topic for patients; tell me why you don't feel comfortable talking about your sexual partners." 
- ☐ E. "This is likely a yeast infection, but I need you to tell me about your sexual relationships to be sure it isn't something more serious." 

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☐  Mark Question

☐ Suggested Highlights

A 23-year-old graduate student comes to the emergency department after being sexually assaulted by a man she met at a bar 5 days ago. The night of the incident, the patient recalls a man introducing himself and handing her a drink. Shortly afterward, she left with him and walked to her apartment, but she is unable to remember the remainder of the night. The following morning, the patient awoke to an empty apartment, and her clothes were strewn across the floor. She felt panicky and decided to take a shower to calm down. The patient mentions that she decided to come in for evaluation after talking to campus crisis services earlier today. She states she has not reported the incident to the authorities due to her hazy memory. Which of the following is the most appropriate initial statement by the physician?

- ☐ A. "Deciding to get an evaluation takes courage; we should perform a pelvic examination today given the circumstances." 
- ☐ B. "I'm sorry that this happened to you; have you taken any emergency contraception?" 
- ☐ C. "It must be difficult talking about what happened; have you been experiencing any nightmares or flashbacks?" 
- ☐ D. "It's important that you feel safe here; please let me know if you need to stop or take a break at any point." 
- ☐ E. "You didn't do anything to deserve this; it's normal to forget details after a trauma and shouldn't prevent you from filing a police report." 

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☐  Mark Question

☐ Suggested Highlights

A 71-year-old woman with metastatic non-small cell lung cancer is admitted to the intensive care unit for malignant airway obstruction with post-obstructive pneumonia and sepsis. Her oncologist calls the admitting physician to provide sign out, saying, "Her disease has been progressing. Our plan was to start a third-line chemotherapy regimen next week." Despite receiving antibiotics and supportive care, she has rapid deterioration of her respiratory status with a very high likelihood of dying in the intensive care unit. When the physician discusses the patient's condition, the patient and family say, "Whatever might have a chance of working, we want everything to be done." Which of the following is the most appropriate response?

- ☐ A. "Given this major change in your clinical status, I want to revisit what you mean by 'everything'." 
- ☐ B. "Have you had discussions with your oncologist about your long-term goals of care?" 
- ☐ C. "I am afraid that continued aggressive treatments could cause more harm than good for you." 
- ☐ D. "I promise we'll do everything we can. I'll arrange for stenting and radiation to open the airway." 
- ☐ E. "Take some time to think things over. I'll return later this evening to continue the conversation." 

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Question Id: 21708



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☐  Mark Question

☐ Suggested Highlights

A 62-year-old man comes to the emergency department with his wife due to back pain. He has had no recent falls or injuries. Medical history includes hypertension and hypercholesterolemia. As the physician is trying to obtain additional history, the patient becomes embarrassed and says that he is having trouble understanding because he has hearing loss, and his hearing aid batteries died a few minutes ago. In addition to closing the door, muting the TV, and turning off nonessential medical equipment to decrease ambient noise, which of the following strategies is the best next step to improve communication with this patient?

- ☐ A. Face the patient directly when talking to him 
- ☐ B. Give the patient a pen and paper to write on 
- ☐ C. Obtain essential history from the wife 
- ☐ D. Speak loudly and slowly in one ear 
- ☐ E. Use a qualified sign language interpreter 

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Question Id: 107420


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☐  Mark Question

☐ Suggested Highlights

A 68-year-old man comes to the office for follow-up of type 2 diabetes mellitus, hypertension, and hyperlipidemia. The patient's diabetes and hypertension are inadequately controlled, requiring multiple modifications to his medication regimen. Extended counseling is provided regarding today's changes and the overall long-term care plan. The patient's next appointment is in a month for a routine health maintenance examination. As the physician is getting ready to leave the room at the end of the scheduled appointment time, the patient states, "Doc, my urinary stream has been slow for the past 6 months, and I've been waking up twice overnight to use the bathroom. Can we talk about this today?" Which of the following is the most appropriate response by the physician?

- ☐ A. "I can evaluate the urinary issue today, but it won't be as thorough because we have limited time." 
- ☐ B. "I can't address the urinary issue today because I have many patients after you. We can discuss it at your next appointment in a month." 
- ☐ C. "It is best to tell me all your concerns at the beginning of the visit so that I can better prioritize our time. At this point, the visit has ended." 
- ☐ D. "It sounds like we should talk about this; however, we are out of time today. Please make another appointment so I can appropriately evaluate you." 

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Question Id: 10289

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☐  Mark Question

☐ Suggested Highlights

A 49-year-old woman is transferred from a community hospital to a tertiary care center for management of an acute exacerbation of chronic obstructive pulmonary disease. The patient receives care overnight from a hospitalist, who transfers care to the teaching service in the morning. The medical team now responsible for the patient meets with her for the first time during morning rounds. The attending physician introduces the team and explains that the patient will be interviewed by the intern. The patient then addresses the intern by his first name and mentions, "You remind me so much of my son. He's very smart and looks a lot like you." Which of the following actions by the intern would be most appropriate to begin the interview?

- ☐ A. Address the patient by her first name and ask about current symptoms 
- ☐ B. Address the patient by her first name and ask if she is comfortable with that 
- ☐ C. Address the patient by her surname and ask about current symptoms 
- ☐ D. Greet the patient and inquire about her preferred form of address 
- ☐ E. Politely ask the patient to use the title of doctor and then inquire about current symptoms 

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Question Id: 11629

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☐  Mark Question

☐ Suggested Highlights

A 45-year-old woman with type 2 diabetes mellitus, obesity, and hypertension comes to the office for a routine follow-up visit. Her medications include a sulfonylurea and ACE inhibitor. The patient has lost 2.3 kg (5 lb) and credits a new energy and weight loss supplement. She purchased the supplement over the Internet on the recommendation of a friend and believes that it contains caffeine and several herbs that promote weight loss. The patient has had no problems tolerating the supplement. Blood pressure is 140/90 mm Hg and pulse is 88/min. Which of the following is the most appropriate response by the physician?

- ☐ A. Advise the patient to avoid the supplement and engage in other forms of weight loss 
- ☐ B. Advise the patient to limit the use of other caffeine-containing products 
- ☐ C. Explain that weight loss due to dietary supplements is unlikely to be sustainable 
- ☐ D. Offer to review the ingredients with the patient and discuss the potential risks of supplements 
- ☐ E. Respect the patient's decision and follow her closely 

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Question Id: 19842

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☐  Mark Question

☐ Suggested Highlights

A male newborn, born at 39 weeks gestation, is evaluated in the labor and delivery unit 2 hours after birth. Prenatal care was limited due to infrequent follow-up, and there were no complications during the delivery. Apgar scores were 7 and 9 at 1 and 5 minutes, respectively. Weight is 2.47 kg (5 lbs, 7 oz) and vital signs are normal. Physical examination shows upslanting palpebral fissures, transverse palmar creasing, and widening of the space between the first and second toes. The parents share that they are glad the delivery went well. Which of the following is the most appropriate statement by the physician at this time?

- ☐ A. "Although your son is healthy and doing well, he may have a chromosomal disorder." 
- ☐ B. "I have some news that may be unexpected; would it be okay to share that with you now?" 
- ☐ C. "This may be difficult to hear, but it looks like your son may have Down syndrome." 
- ☐ D. "Unfortunately, your son may have Down syndrome; I'd like to do some bloodwork to confirm." 
- ☐ E. "Your son has some concerning examination findings; did you have any prenatal testing done?" 

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Question Id: 21167



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☐ Mark Question

☐ Suggested Highlights

A 45-year-old woman comes to the office for a routine follow-up. The patient feels well overall but mentions that climbing the last flight of stairs to her 3rd-floor apartment has become harder. She says, "Sometimes I have to stop at the landing to catch my breath." Medical history includes cocaine use disorder in remission for the past 4 years; it is otherwise unremarkable. The patient does not use alcohol or illicit drugs. She has a 25-pack-year smoking history and has been counseled to quit smoking in the past. The patient says, "I've tried to quit several times. I can cut down to a few cigarettes or half a pack for a day or two, but then it's just too difficult." Which of the following is the most appropriate response by the physician regarding this patient's smoking?

- ☐ A. "I'm concerned that continuing to smoke is contributing to your breathing problems."
- ☐ B. "Quitting on your own is difficult and medication can help. Would you consider this?"
- ☐ C. "Since you stopped using cocaine, I'm hopeful that you can eventually stop smoking cigarettes."
- ☐ D. "Stopping smoking will likely improve your breathing and decrease your risk for other conditions."
- ☐ E. "You were successful in stopping cocaine. How might your experience help you to quit smoking?"

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